

§ PATIENT AND MEMBER 360 COPILOT

Docket

Every AI claim goes on trial. If a record doesn't back it up, it is struck.

THE PROBLEM

Care teams can't act on an answer they can't check

Siloed records

EHR, medical claims and pharmacy claims each tell part of the story, and they often disagree.

Dense documents

The deciding facts sit in notes, coverage policies and drug labels, not in a table.

New rules, now

Since Jan 2026, CMS-0057-F requires prior auth decisions in 7 days (72 hours if urgent) and a specific reason for every denial.

THE IDEA

Every question is tried like a court case

ADVOCATE

An agent argues for the motion, citing record IDs.

CHALLENGER

An agent argues against it from the same evidence.

CLERK

A rule-based tool checks each line against its source and strikes the rest.

JUDGE

Rules only on what survives, and says what would change the ruling.

The Judge can grant, hold or send back for records. It never denies; that stays with a licensed reviewer.

The Clerk's four objections

Objection	Test	Effect
Hearsay	The statement cites no record	Struck
Not supported	The cited record doesn't contain the claimed fact	Struck
Stale	Evidence is outside the policy's time window	No weight
Conflict	Two records disagree about the same fact	Blocks approval

Numbers and dates must match the source exactly. Wording is checked with an entailment prompt. Risk uses the published LACE score, where every point traces to a record.

A heart-failure drug request that looks approvable

Advocate: LVEF 30% on 16 Sep, NYHA III, cardiology prescriber. All three criteria met.

~~Advocate: She has tolerated ARB therapy before.~~

Struck, hearsay: no record cited; no ARB fills in 36 months.

Challenger: Lisinopril was refilled after discharge told her to stop it. The label requires a 36-hour gap.

Challenger: A 2024 ER note records swelling after lisinopril; the allergy list says none.

Two conflicts logged

RULING

Safety hold: route to pharmacist

Criteria are met, but both conflicts touch the label's contraindications.

It drafts the specific reason and opens a Jira review before the 9 Oct deadline.

THREE CASES, THREE OUTCOMES

Each ruling traces back to records

Case	Ruling	Deciding evidence
Care management enrollment	Granted	LACE 14 (threshold 10); new diuretic dose unfilled for 15 days; lives alone
Heart-failure drug	Safety hold	Criteria met; open conflicts on lisinopril use and a possible reaction
Lumbar MRI	Needs records	23 of 42 days of therapy; "progressive" claim struck as not supported

Every ruling also states what would change it, for example therapy notes through 20 Oct for the MRI.

Built end to end inside Snowflake

Sources

Synthetic EHR, claims, pharmacy, PA requests, notes, policies, labels

Unify

Streams, dynamic tables and Member 360; AI_PARSE_DOCUMENT for PDFs

Retrieve

Semantic view for Cortex Analyst; Cortex Search over documents

Trial

Cortex Agents for Advocate, Challenger and Judge; Python UDF Clerk

Act

Streamlit app; Jira and Slack via MCP; nightly task re-tries changes

When a new claim or fill arrives, the nightly task re-tries that member's open cases. That's how the lisinopril refill was caught.

Built through CoCo from plan to test

Planning

Profiled the data and wrote the ontology and design doc

Development

Generated synthetic data, Member 360 views, LACE, search, Clerk UDF

Execution

Stream plus a nightly task at 02:00 IST that re-tries changed members

Testing

Golden questions; a fake citation must be struck; LACE must equal 14

Mapped to the judging criteria

Real-world relevance

Specific reasons and the 7-day clock match CMS-0057-F, live since January 2026.

Technical execution

Multi-agent debate plus a deterministic validator; structured and unstructured data joined in one view.

Completeness

Member 360, cited Q&A, risk stratification, actions through MCP and a scheduled run.

No opaque predictions: when evidence is missing, Docket says so instead of guessing.

No source, no claim.

Try the live demo: open a case, click any citation, and ask the court a question.

[Demo link] · [Team name]